

Public Policy for the Poor? A Randomized Evaluation of the Mexican Universal Health Insurance Program

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(Talk at Harvard School of Public Health, 4/21/2010)

Project References

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- Gary King et al., A 'Politically Robust' Experimental Design for Public Policy Evaluation, with Application to the Mexican Universal Health Insurance Program *Journal of Policy Analysis and Management*, 26, 3 (2007): 479-506.

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- <http://gking.harvard.edu/projects/mex.shtml>

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 - uses data far more efficiently to **find effects** and **save money**

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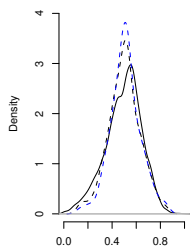
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- Imai-King-Nall: prove above results and offer simple estimators for MPDs making minimal assumptions for both **intent to treat** and **complier average treatment** effects

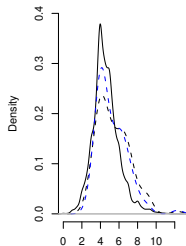
Remaining in study: 148 clusters (74 pairs) in 7 states



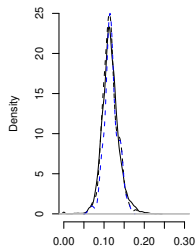
Clusters are Representative On Measured Variables



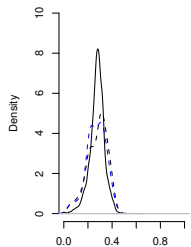
Prop earning <2 min wages



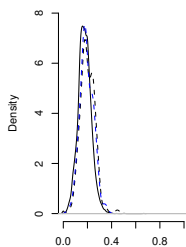
Mean Years Education



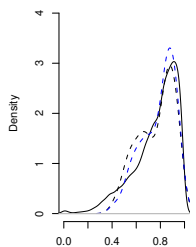
Prop aged 0-4 years old



Prop Employed

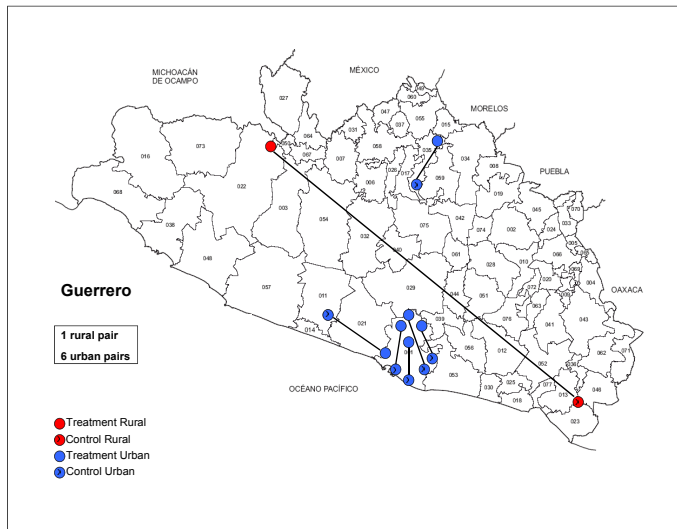


Prop Female-headed HH



Prop w/o Soc Sec Rights

Matched Pairs, Guerrero

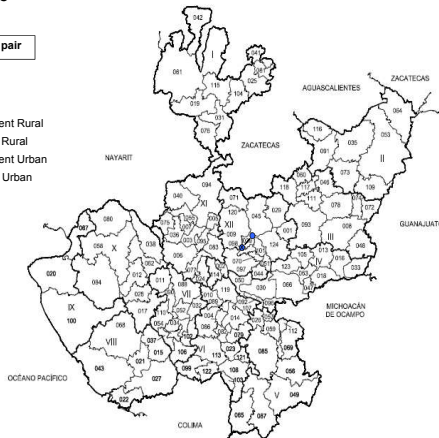


Matched Pairs, Jalisco

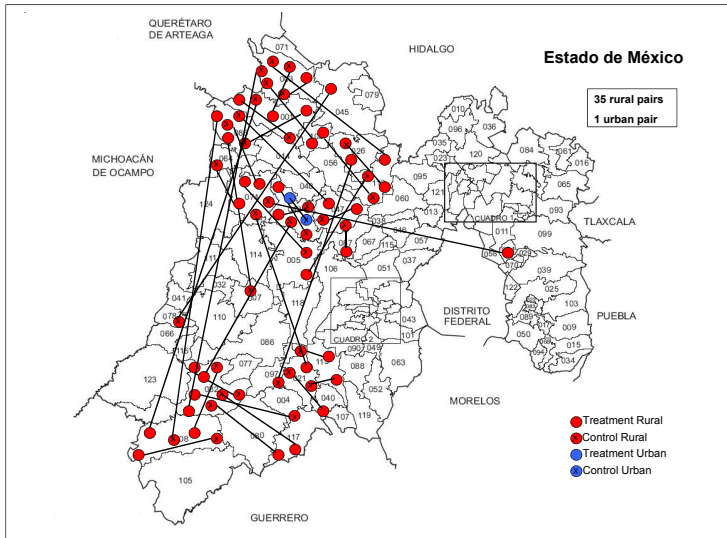
Jalisco

1 urban pair

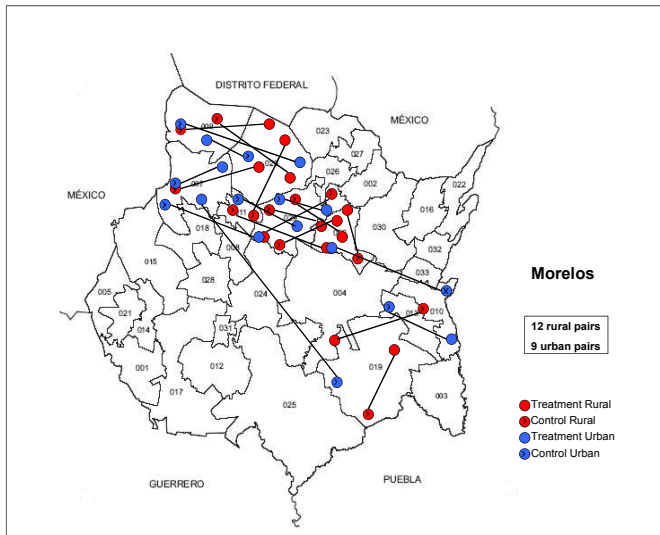
- Treatment Rural
- Control Rural
- Treatment Urban
- Control Urban



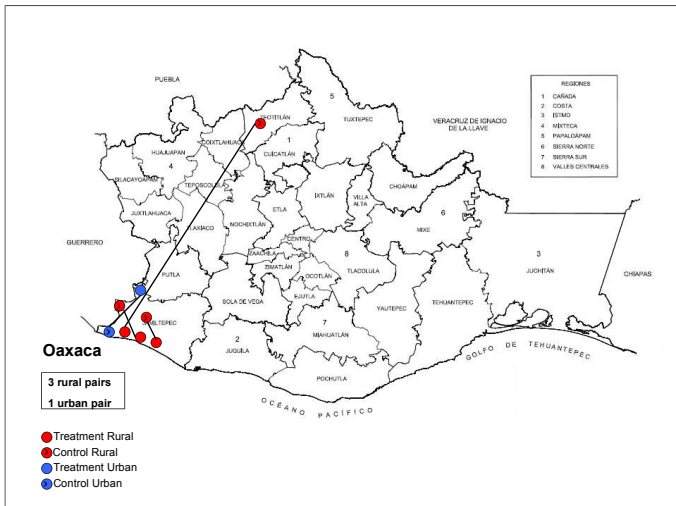
Matched Pairs, Estado de México



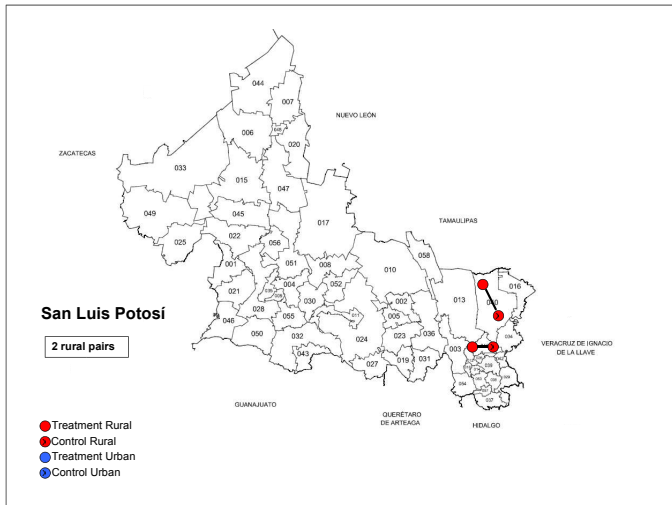
Matched Pairs, Morelos



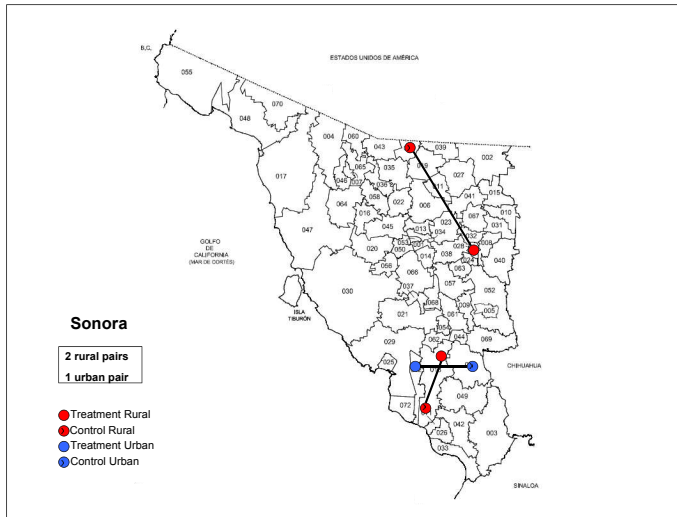
Matched Pairs, Oaxaca



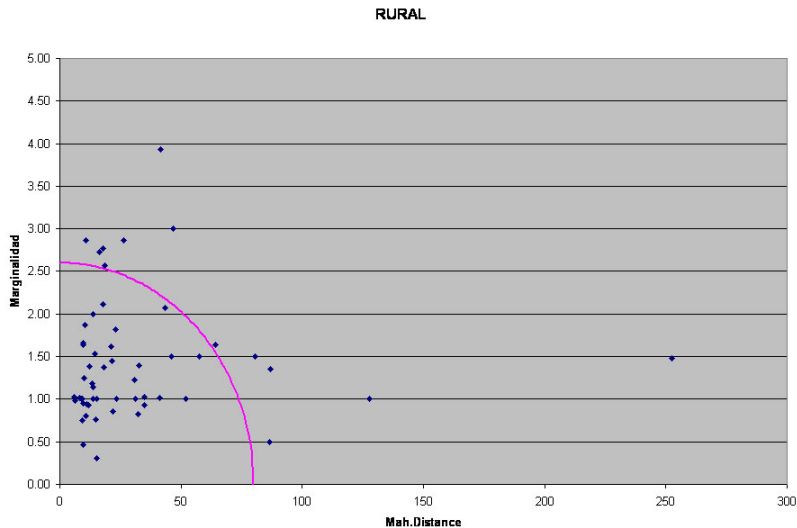
Matched Pairs, San Luis Potosí



Matched Pairs, Sonora



Choosing Pairs for the Survey



Design and Analysis Strategy is Triply Robust

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Design has three parts

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If matching **or** randomization **or** statistical analysis is right, but the other two are wrong, results are still unbiased

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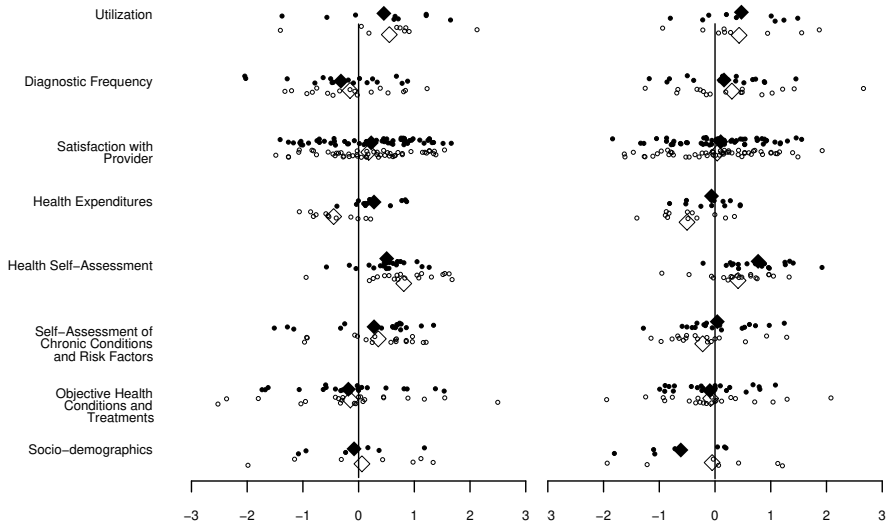
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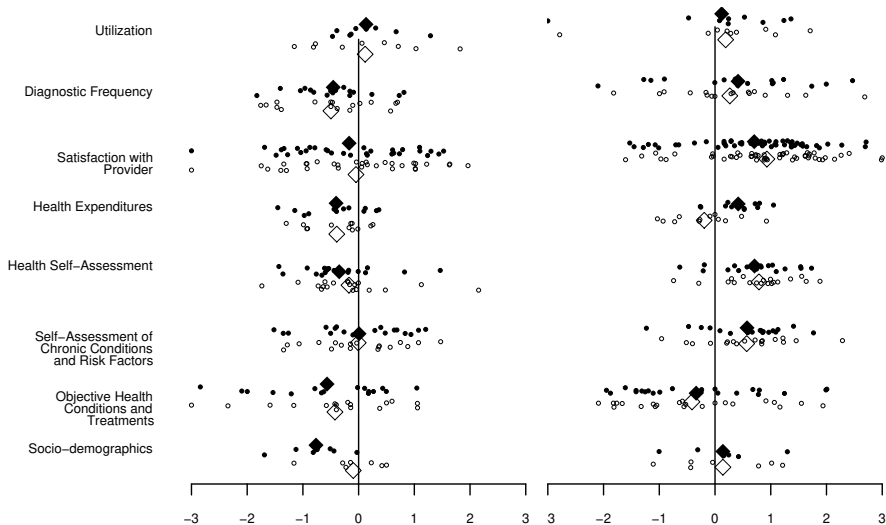
Two Additional Checks if Triple Robustness Fails

- 1 If one of the three works, then “effect of SP” on time 0 outcomes (measured in baseline survey) must be zero
- 2 If we lose pairs, we check for selection bias by rerunning this check

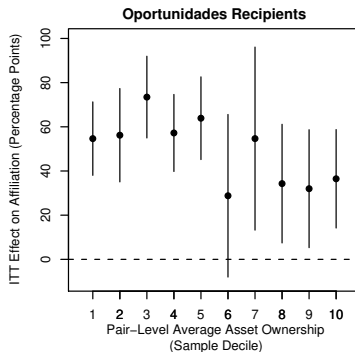
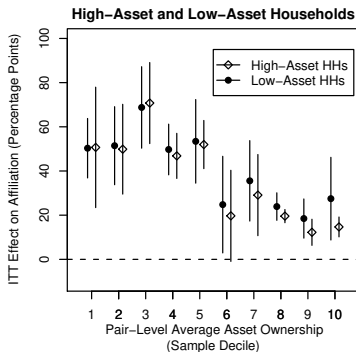
ITT on Outcome Measures at Baseline, for all families (left) and poor families, in Oportunidades (right)



ITT on Outcome Measures at Baseline, for wealthy families (left) and middle income families (right)



Effect of Encouragement on Seguro Popular Affiliation



Horizontal axes: per-capita asset ownership deciles of areas (poorer to the left). Vertical axes: percentage point causal effect of encouragement to affiliate on Seguro Popular affiliation.

Poor areas, not poor households, are affiliated the most

Effect on % of Households with Catastrophic Health Expenditures

	All Study Participants			Experimental Compliers		
	Average (Control)	ITT	SE	Average (Control)	CACE	SE
All	8.4	1.9*	(.9)	9.5	5.2*	(2.3)
Low Asset	9.9	3.0*	(1.3)	11.0	6.5*	(2.5)
High Asset	7.1	0.9	(0.8)	7.9	3.0	(2.7)
Female-Headed	8.5	1.4	(1.1)	10.6	3.8	(3.0)

“Catastrophic expenditures”: out-of-pocket health expenses $> 30\%$ of post-subsistence income

Effect on Out-of-pocket Health Expenditures, I (in pesos)

	All Study Participants			Experimental Compliers		
	Average (Control)	ITT	SE	Average (Control)	CACE	SE
Overall:						
All	\$1631.3	\$258.0	(\$175)	\$1712.7	\$689.7	(\$453)
Low Asset	1360.2	425.6*	(197)	1502.6	915.3*	(392)
High Asset	1867.9	128.4	(201)	1933.2	428.2	(669)
Female-Headed	1509.1	156.5	(207)	1689.9	428.6	(566)
Inpatient Care:						
All	532.5	96.9*	(44)	557.1	259.1*	(112)
Low Asset	527.1	188.2*	(73)	579.0	404.8*	(142)
High Asset	537.2	31.1	(52)	536.2	103.6	(173)
Female-Headed	452.5	115.1*	(68)	510.0	315.2*	(182)
Outpatient Care:						
All	448.3	116.7*	(63)	499.1	312.0*	(161)
Low Asset	412.3	176.7*	(73)	466.3	380.0*	(147)
High Asset	479.7	81.9	(69)	533.0	272.9	(230)
Female-Headed	416.3	110.4	(75)	496.8	302.4	(202)

Effect on Out-of-pocket Health Expenditures, II (in pesos)

	All Study Participants			Experimental Compliers		
	Average (Control)	ITT	SE	Average (Control)	CACE	SE
Medicine:						
All	521.1	20.0	(41)	534.5	53.3	(109)
Low Asset	427.3	17.8	(46)	444.7	38.3	(100)
High Asset	603.0	29.4	(47)	627.5	98.1	(157)
Female-Headed	625.6	53.6	(55)	738.9	146.8	(151)
Medical Devices:						
All	139.7	-8.8	(23)	117.8	-23.4	(62)
Low Asset	72.0	-0.2	(20)	72.8	-0.5	(43)
High Asset	198.8	-16.5	(29)	165.6	-55.1	(98)
Female-Headed	155.5	10.9	(34)	162.8	30.0	(94)

Utilization: Overall

	All Study Participants			Experimental Compliers		
	Average (Control)	ITT	SE	Average (Control)	CACE	SE
Utilization (Procedures):						
Used Outpatient Services (%)	62.6	-1.5	(1.9)	64.8	-4.0	(5.2)
Outpatient Visits (count)	1.6	-0.03	(0.09)	1.7	-0.08	(0.23)
Hospitalized (%)	7.6	-0.2	(0.5)	7.9	-0.5	(1.5)
Hospitalizations (count)	0.1	-0.003	(0.006)	0.1	-0.01	(0.02)
Satisfaction with Provider (%)	68.0	-1.0	(1.6)	69.8	-2.6	(4.5)
Utilization (Preventative) (%):						
Eye Exam Last Yr.	10.0	-0.7	(0.7)	9.8	-1.8	(1.9)
Flu Vaccine	25.7	-1.8	(1.4)	27.2	-4.9	(3.7)
Mammogram Last Yr.	5.1	-0.9	(0.6)	5.2	-2.3	(1.6)
Cervical Last Yr.	21.8	-1.3	(2.0)	22.2	-3.2	(4.8)
Pap Test Last Yr.	31.9	-2.3	(2.1)	33.2	-5.8	(5.0)

Self-Assessment: Overall

	All Study Participants			Experimental Compliers		
	Average (Control)	ITT	SE	Average (Control)	CACE	SE
Overall Health	55.7	4.2*	(2.0)	54.3	8.9*	(3.9)
Mobility	86.7	1.0	(1.0)	86.3	2.1	(2.0)
Vigorous Activity	69.2	4.6*	(2.7)	67.9	9.8*	(5.7)
Self-Care	95.3	0.4	(0.6)	95.2	0.8	(1.2)
Soreness	80.3	2.6*	(1.5)	79.3	5.5*	(3.1)
Pain	82.4	2.4*	(1.4)	81.4	5.2*	(2.8)
Sleeping	85.1	2.7*	(1.3)	84.3	5.9*	(2.5)
Depression	77.3	6.4*	(3.7)	76.0	13.8*	(7.3)
Anxiety	85.9	3.1	(2.0)	85.2	6.7	(4.1)

Self-Assessment, Controlling for Baseline Levels

	ITT		CACE	
Overall Health	0.6	(2.2)	1.7	(6.0)
Mobility	0.2	(0.9)	0.6	(2.5)
Vigorous Activity	3.3	(2.4)	8.9	(6.4)
Self-Care	-0.2	(0.6)	-0.5	(1.6)
Soreness	1.0	(1.4)	2.6	(3.8)
Pain	1.1	(1.2)	3.0	(3.3)
Sleeping	1.0	(1.0)	2.6	(2.5)
Depression	0.6	(3.0)	1.5	(7.9)
Anxiety	0.8	(1.8)	2.1	(4.8)

A **difference-in-difference test**: The causal effect of Seguro Popular on the change from baseline to followup in the difference between treated and control groups on health self-assessment variables.

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For more information

<http://GKing.Harvard.edu>